This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Shingles (Herpes Zoster) Treatment

Aciclovir, Valaciclovir or Famciclovir

Patient Group Direction, version 009, issued 30 September 2026. Supersedes Version 008, 14 September 2026.

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This PGD is for the TREATMENT of active shingles with oral antivirals. It is not a vaccination PGD; Shingrix is covered by the separate Shingles vaccination PGD.

Note on NHS Pharmacy First. Shingles is a commissioned Pharmacy First pathway in England with national PGDs for aciclovir and valaciclovir. Where a patient is eligible for that service it should be used, and this PGD should not displace it. This PGD exists for private supply, for patients outside NHS eligibility, and to cover famciclovir, which the national service does not include.

Summary of Green Book, NICE CKS and SmPC guidance for oral antiviral treatment of herpes zoster (shingles) in adults

Immunisation against infectious disease (the Green Book), chapter 28a Shingles (herpes zoster), UK Health Security Agency, chapter dated 12 August 2025, page last updated 19 August 2025. NICE Clinical Knowledge Summary: Shingles (Scenario: Management), last revised January 2026. Aciclovir 800mg Tablets, Summary of Product Characteristics, Wockhardt UK Ltd (PL 29831/0003), text revised 14 April 2026, emc updated 27 April 2026. Valtrex 500 mg film-coated tablets (valaciclovir), Summary of Product Characteristics, GlaxoSmithKline UK (PL 00003/0352), text revised 18 June 2026, emc updated 30 June 2026. Famvir 500 mg film-coated tablets (famciclovir), Summary of Product Characteristics, Phoenix Labs (PL 35104/0027), text revised 18 March 2019, emc updated 27 August 2020. Summarised 10 September 2026; famciclovir added 11 September 2026.

The disease and how it presents

The Green Book states that shingles is caused by reactivation of latent varicella zoster virus, generally decades after primary infection, and that reactivation is associated with immune senescence or suppression of the immune system, for example immunosuppressive therapy, HIV infection or malignancy.

The risk and severity of shingles increase with age; the Green Book gives an estimated lifetime risk of one in four and, before vaccination, an annual incidence of around 790 to 880 cases per 100,000 in people aged 70 to 79 in England and Wales.

The Green Book describes the first signs as abnormal skin sensations and pain in the affected dermatome, with headache, photophobia, malaise and less commonly fever in the prodromal phase.

Within days or weeks a unilateral vesicular rash typically appears in a dermatomal distribution; in immunosuppressed individuals a rash involving multiple dermatomes may occur.

The affected area may be intensely painful with associated paraesthesia, intense itching is common, and the rash typically lasts between two and four weeks.

The Green Book also notes an increased risk of shingles in people with systemic lupus erythematosus, rheumatoid arthritis, diabetes and Wegener's granulomatosis.

Who benefits from antivirals and the treatment window

NICE CKS recommends offering oral aciclovir, valaciclovir or famciclovir within 72 hours of rash onset to people aged 50 years and over, people with non-truncal involvement (excluding the head and neck, where admission or specialist advice is indicated), people with moderate or severe pain or moderate or severe rash, people with predisposing skin conditions, and immunocompromised people where clinical judgement supports oral treatment (for example non-severe immunocompromise, localised rash, no eye involvement, not systemically unwell and close follow-up possible).

NICE CKS says to consider antiviral treatment for other people aged under 50 with shingles of the extremities or trunk depending on clinical judgement.

If treatment cannot be started within 72 hours, NICE CKS says to consider starting it up to one week after rash onset, especially where the person is at higher risk of severe shingles or complications (continued vesicle formation, signs of cutaneous, visceral or neurological dissemination, older age, immunocompromise or severe pain).

NICE CKS states that antiviral treatment is not recommended in immunocompetent children.

The valaciclovir SmPC states that treatment should start as soon as possible after diagnosis and that there are no data on treatment started more than 72 hours after onset of the zoster rash.

The aciclovir SmPC states that treatment of herpes zoster yields better results if initiated as soon as possible after the onset of the rash.

The valaciclovir SmPC states that in immunocompromised patients antiviral treatment is suggested for those presenting within one week of vesicle formation or at any time before full crusting of lesions.

The valaciclovir SmPC states that valaciclovir accelerates resolution of pain, reduces the duration and proportion of patients with zoster-associated pain (including post-herpetic neuralgia in patients older than 50), and reduces the risk of ocular complications of ophthalmic zoster.

Doses and course lengths

The aciclovir SmPC dose for treatment of herpes zoster in adults is 800 mg five times daily at approximately four-hourly intervals, omitting the night-time dose, for seven days.

Aciclovir tablets may be dispersed in a minimum of 50 ml of water or swallowed whole with a little water, and patients on high doses should be adequately hydrated.

The valaciclovir SmPC dose for herpes zoster and ophthalmic zoster in immunocompetent adults is 1000 mg three times daily for seven days (3000 mg total daily dose).

For immunocompromised adults the valaciclovir dose is 1000 mg three times daily for at least seven days and for 2 days following crusting of lesions.

The famciclovir SmPC dose for herpes zoster and ophthalmic zoster in immunocompetent adults is 500 mg three times daily for seven days, and for herpes zoster in immunocompromised adults 500 mg three times daily for ten days; in both cases treatment should be initiated as soon as possible after diagnosis. Famciclovir can be taken without regard to meals.
The famciclovir SmPC indication covers the treatment of herpes zoster and ophthalmic zoster in immunocompetent adults and of herpes zoster in immunocompromised adults; the SmPC states that its safety and efficacy in children and adolescents under 18 have not been established.
The valaciclovir SmPC indication covers herpes zoster and ophthalmic zoster in immunocompetent adults and herpes zoster in adults with mild or moderate immunosuppression; the aciclovir SmPC indication is treatment of herpes zoster infections.
Both SmPCs state that in severely immunocompromised patients (for example after marrow transplant) or in patients with impaired absorption from the gut, intravenous dosing should be considered.

Renal adjustment

The aciclovir SmPC states that aciclovir is eliminated by renal clearance so the dose must be adjusted in renal impairment, and that elderly patients are likely to have reduced renal function so the need for dose adjustment must be considered.
For herpes zoster the aciclovir SmPC recommends 800 mg three times daily at approximately eight-hourly intervals for moderate renal impairment (creatinine clearance 10 to 25 ml/minute) and 800 mg twice daily at approximately twelve-hourly intervals for severe renal impairment (creatinine clearance below 10 ml/minute).
The valaciclovir SmPC table for treatment of herpes zoster gives 1000 mg three times daily for creatinine clearance 50 ml/min or above, 1000 mg twice daily for 30 to 49 ml/min, 1000 mg once daily for 10 to 29 ml/min, and 500 mg once daily below 10 ml/min; on intermittent haemodialysis the dose should be given after dialysis.
Both SmPCs state that adequate hydration should be maintained, that the risk of renal impairment is increased by concomitant nephrotoxic drugs, and that elderly patients and patients with renal impairment are at increased risk of neurological side effects and should be closely monitored.
The valaciclovir SmPC lists nephrotoxic medicines requiring caution and renal monitoring, including aminoglycosides, organoplatinum compounds, iodinated contrast media, methotrexate, pentamidine, foscarnet, ciclosporin and tacrolimus.
Both SmPCs state that cimetidine and probenecid, and other drugs competing for active renal tubular secretion, increase aciclovir exposure; the aciclovir SmPC says no dosage adjustment is necessary because of the wide therapeutic index, while the valaciclovir SmPC advises caution at zoster treatment doses.
The valaciclovir SmPC states that dose modification is not required in mild or moderate cirrhosis.
The famciclovir SmPC states that reduced clearance of penciclovir is related to reduced renal function as measured by creatinine clearance, and its table for herpes zoster gives the full dose of 500 mg three times daily only where creatinine clearance is 60 ml/min or above; below that it gives 500 mg twice daily for 40 to 59 ml/min, 500 mg once daily for 20 to 39 ml/min and 250 mg once daily below 20 ml/min, each for 7 days in immunocompetent and 10 days in immunocompromised adults, with 250 mg after each dialysis for haemodialysis patients.
The famciclovir SmPC states that in the elderly (65 years and over) dose modification is not required unless renal function is impaired, that penciclovir exposure is about 30% higher in older volunteers, and that acute renal failure has been reported rarely in patients with underlying renal disease where the dose was not reduced for the level of renal function.
The famciclovir SmPC states that concurrent probenecid may increase penciclovir concentrations, so patients taking 500 mg three times daily with probenecid should be monitored for toxicity, and that raloxifene, a potent inhibitor of the aldehyde oxidase that converts famciclovir to penciclovir, could affect efficacy, so the clinical response should be monitored when the two are co-administered.
The famciclovir SmPC states that no dose adjustment is required in mild or moderate hepatic impairment, and that famciclovir has not been studied in severe hepatic impairment, where conversion to penciclovir may be impaired and efficacy may be reduced.

Contraindications and warnings

Both SmPCs contraindicate use in patients hypersensitive to aciclovir or valaciclovir or to any excipient.
Both SmPCs warn of severe cutaneous adverse reactions (toxic epidermal necrolysis, Stevens-Johnson syndrome, acute generalised exanthematous pustulosis, drug reaction with eosinophilia and systemic symptoms, erythema multiforme); treatment must be stopped immediately if suggestive signs appear and patients who have had such a reaction with aciclovir or valaciclovir must not receive either drug again.
The famciclovir SmPC contraindicates use in patients hypersensitive to famciclovir, to penciclovir or to any excipient. Its post-marketing reactions of unknown frequency include serious skin reactions (erythema

multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis), hypersensitivity vasculitis, anaphylactic reaction and seizure, and it advises that patients who experience dizziness, somnolence, confusion or other central nervous system disturbances should not drive or operate machinery.
The aciclovir SmPC advises caution in patients with underlying neurological abnormalities, severe hepatic or electrolyte abnormalities, or significant hypoxia, and notes the tablets contain lactose.
The aciclovir SmPC states that caution should be exercised in pregnancy by balancing potential benefits against any possible hazard, noting that the pregnancy registry has not shown an increase in birth defects; the valaciclovir SmPC says valaciclovir should only be used in pregnancy if the potential benefits outweigh the potential risk.
The aciclovir SmPC advises caution in nursing women because aciclovir is detected in breast milk; the valaciclovir SmPC says valaciclovir should be used with caution during breastfeeding and only when clinically indicated.
The famciclovir SmPC states that limited data (fewer than 300 pregnancy outcomes) have not indicated a specific foetal defect or congenital anomaly, that famciclovir should only be used in pregnancy when the potential benefits outweigh the potential risks, and that it is unknown whether famciclovir is excreted in human breast milk; if the woman's condition mandates treatment, discontinuation of breastfeeding may be considered.
NICE CKS says to seek specialist advice before prescribing antiviral treatment to a pregnant or breastfeeding woman with shingles.
Common adverse effects in the aciclovir SmPC are headache, dizziness, nausea, vomiting, diarrhoea, abdominal pain, pruritus, rashes including photosensitivity, fever and fatigue; the valaciclovir SmPC lists headache as very common and nausea, dizziness, vomiting, diarrhoea, rashes and pruritus as common.
The famciclovir SmPC lists headache as very common and dizziness, nausea, vomiting, abdominal pain, diarrhoea, abnormal liver function tests, rash and pruritus as common, with confusional state and somnolence (predominantly in the elderly), angioedema and urticaria uncommon, and hallucinations, palpitations, thrombocytopenia and cholestatic jaundice rare.
Both SmPCs describe reversible neurological reactions (confusion, hallucinations, agitation, tremor, convulsions, encephalopathy, coma) usually in patients with renal impairment or other predisposing factors, and renal impairment or acute renal failure that may be associated with crystalluria.

Referral: ophthalmic, immunocompromised, disseminated and pregnancy

NICE CKS says to admit to hospital or seek immediate specialist advice for shingles in the ophthalmic distribution of the trigeminal nerve, especially with Hutchinson's sign (rash on the tip, side or root of the nose), eye pain, photophobia, reduced corneal sensitivity, visual impairment or an unexplained red eye.
NICE CKS also says to admit or seek immediate specialist advice where serious complications such as meningitis, encephalitis or myelitis are suspected, where the head and neck are affected especially in elderly people, where there are haemorrhagic or necrotic lesions, multisegmental involvement, satellite lesions, mucosal involvement or generalised zoster, where the person is severely immunosuppressed, where an immunocompromised child has shingles, or where there are signs of visceral or central nervous system involvement.
NICE CKS says severely immunocompromised people should be referred for intravenous aciclovir.
The valaciclovir SmPC states that patients with complicated herpes zoster (visceral involvement, disseminated zoster, motor neuropathies, encephalitis and cerebrovascular complications) should be treated with intravenous antiviral therapy, as should immunocompromised patients with ophthalmic zoster or a high risk of dissemination and visceral involvement.
The valaciclovir SmPC says clinical response should be closely monitored, particularly in immunocompromised patients, with intravenous therapy considered when response to oral therapy is insufficient.
The famciclovir SmPC carries the same warnings for zoster treatment: clinical response should be closely monitored, particularly in immunocompromised patients, with intravenous antiviral therapy considered when response to oral therapy is insufficient; patients with complicated herpes zoster (visceral involvement, disseminated zoster, motor neuropathies, encephalitis and cerebrovascular complications), and immunocompromised patients with ophthalmic zoster or a high risk of dissemination and visceral involvement, should be treated with intravenous antiviral therapy.
NICE CKS says to refer or seek specialist advice if new vesicles are forming after 7 days of oral antiviral treatment or healing is delayed, if a person thought to be immunocompetent has had two episodes of shingles, if shingles recurs in an immunocompromised person, or if pain is inadequately controlled by oral analgesia or a strong opioid is being considered.
The Green Book states that ophthalmic zoster occurs in 10 to 20% of shingles cases, that complications depend on the nerves affected and include paresis, facial palsy and herpes zoster ophthalmicus with keratitis, corneal ulceration, conjunctivitis, retinitis, optic neuritis or glaucoma, and that disseminated disease (pneumonia,

hepatitis, encephalitis, disseminated intravascular coagulopathy) is more likely in severely immunosuppressed people with a case fatality of 5 to 15%.

Pain and post-herpetic neuralgia in outline

The Green Book defines post-herpetic neuralgia as pain that persists for, or appears more than, 90 days after onset of the rash; it is seen more frequently in older people and on average lasts three to six months but can persist longer.

The Green Book gives pre-vaccine estimates of the proportion developing post-herpetic neuralgia after 90 days rising from 9% at age 60 to 64 to 52% at age 85 and over.

NICE CKS recommends paracetamol or an NSAID such as ibuprofen, alone or with a weak opioid such as codeine, for mild to moderate pain in adults, and a choice of amitriptyline (off-label), duloxetine (off-label), gabapentin or pregabalin for moderate to severe pain.

NICE CKS says capsaicin cream may be considered for localised neuropathic pain where oral treatment is to be avoided or is not tolerated, and that oral corticosteroids may be considered in the first 2 weeks after rash onset in immunocompetent adults with localised shingles and severe pain, but only in combination with antiviral treatment.

NICE CKS says to seek specialist advice or refer to the pain team if pain is inadequately controlled by oral analgesia or a strong opioid is being considered, and to see its post-herpetic neuralgia topic for pain persisting after the skin lesions have resolved.

Infectivity advice

The Green Book states that individuals with active lesions, particularly if immunosuppressed, can transmit varicella zoster virus to susceptible individuals to cause chickenpox, and that there is no evidence that shingles can be acquired from another individual who has chickenpox.

NICE CKS says the person is infectious until all the vesicles have crusted over, approximately 7 days after rash onset.

NICE CKS advises avoiding skin contact with people at high risk of complications (immunocompromised people, newborn babies and elderly people) and with people who have not had chickenpox (especially pregnant women) until the rash crusts over.

NICE CKS advises avoiding sharing clothes and towels, avoiding touching or scratching the rash, washing hands often, wearing loose-fitting clothes, covering lesions not under clothes while the rash is weeping, keeping the rash clean and dry, and avoiding topical antibiotics and adhesive dressings.

NICE CKS advises avoiding work, school or daycare if the rash is weeping and cannot be covered, and seeking medical advice if temperature rises because this may indicate bacterial infection.

The Green Book states that at-risk individuals who have had a significant exposure to shingles require post-exposure management under the varicella chapter and the UKHSA post-exposure prophylaxis guidelines, and that the shingles vaccine is not recommended for the treatment of shingles or post-herpetic neuralgia.

What the sources do not say

The Green Book chapter is an immunisation chapter and does not give antiviral doses or the 72-hour treatment window; those come from NICE CKS and the SmPCs.

The aciclovir SmPC does not state the 72-hour window explicitly; it says treatment yields better results if started as soon as possible after rash onset.

NICE CKS does not give the antiviral doses or renal dose adjustments in its management scenario; those are taken from the three SmPCs.

None of the three SmPCs gives a specific age threshold (such as 50 years) for deciding who should receive treatment; that threshold comes from NICE CKS.

The famciclovir SmPC gives no treatment window in hours; it says treatment should be initiated as soon as possible after diagnosis. It does not list drug reaction with eosinophilia and systemic symptoms (DRESS) among famciclovir's adverse reactions; the adverse effects row and the exclusion for a previous DRESS reaction in this PGD are retained as the conservative position.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in the recognition of herpes zoster and, critically, in distinguishing it from the presentations that require urgent referral rather than supply • All users must be able to assess pain using a validated 0 to 10 scale (this PGD treats a score of 4 or more as moderate or severe pain and 7 or more as severe pain) and to recognise the features of ophthalmic involvement, including Hutchinson's sign • All users must be familiar with the definition of severe immunosuppression in Green Book chapter 28a • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Treatment of uncomplicated herpes zoster (shingles) in immunocompetent adults, and in adults with non-severe (mild or moderate) immunosuppression using valaciclovir or famciclovir, to reduce the severity and duration of the episode and to reduce the risk of post-herpetic neuralgia. Severe immunosuppression as defined in Green

	Book chapter 28a is excluded.
Inclusion criteria	• Adults aged 18 years and over. • A clinical presentation consistent with shingles: a unilateral, dermatomal, painful vesicular rash that does not cross the midline, with or without a preceding prodrome of pain, itching or tingling. • Rash onset within 72 hours, AND at least one of: aged 50 years or over; non-truncal involvement of the limbs or perineum (sacral dermatomes, including the buttocks, count as truncal; head or neck involvement excludes: see below); moderate or severe pain, meaning a score of 4 or more on a 0 to 10 scale; or moderate or severe rash with confluent lesions. • OR rash onset within 7 days, AND at least one of: continued formation of new vesicles; severe pain, meaning a score of 7 or more on a 0 to 10 scale; age 70 or over; or a high risk of severe shingles, for example severe atopic eczema. • Valid informed consent obtained. • No exclusion criteria or red flags present.
Red flags requiring urgent referral rather than supply	• Ophthalmic involvement. Rash in the distribution of the ophthalmic division of the trigeminal nerve, any visual symptom, an unexplained red eye, or Hutchinson's sign, which is a rash on the tip, side or root of the nose and predicts subsequent eye inflammation. Refer the same day for ophthalmology assessment. • Ramsay Hunt syndrome or facial nerve involvement. Rash in or around the ear, hearing loss, vertigo, altered taste, or unilateral facial weakness. Refer urgently. • Signs of meningitis: neck stiffness, photophobia, mottled skin. Refer to A&E. • Head or neck involvement, including the face, scalp, ear or eye. Refer or seek specialist advice the same day (NICE CKS), urgently where the eye may be involved. • Signs of encephalitis: disorientation, confusion, change in behaviour. Refer to A&E. • Signs of myelitis: muscle weakness, loss of bladder or bowel control. Refer to A&E. • Disseminated or widespread rash, or rash crossing the midline, which suggests dissemination. • Severe immunosuppression, as defined in Green Book chapter 28a. Refer to A&E if the rash is widespread or severe, or the patient is systemically unwell. • Any sign of sepsis or serious systemic infection. Call 999. • Pain not controlled by over-the-counter analgesia. Refer to a prescriber the same day. • Systemic illness not meeting the threshold for sepsis. Refer to a prescriber the same day.
Exclusion criteria	• Under 18 years of age. • Pregnancy, known or suspected. Refer to a prescriber. • Breastfeeding. Refer to a prescriber. NICE CKS advises specialist advice before antiviral treatment in a breastfeeding woman. • Rash onset more than 7 days ago. Refer for a prescriber decision, which may still favour treatment where new vesicles are forming

	or pain is severe. ● Severe immunosuppression as defined in Green Book chapter 28a. See the red flags above. ● Renal impairment below the thresholds in the dosing section. Refer rather than attempting to operate a renal dosing ladder in the pharmacy. ● Known hypersensitivity to the antiviral to be supplied. Aciclovir and valaciclovir are cross-reactive; famciclovir and penciclovir are cross-reactive. ● Any previous DRESS reaction to valaciclovir or famciclovir. These must never be restarted. ● Concurrent ciclosporin, tacrolimus, mycophenolate, aminophylline or theophylline. Refer to a prescriber. ● Inability to swallow or absorb oral medication. ● Current long-term prophylactic treatment with the same class of antiviral. ● Any underlying neurological condition. ● Unable to maintain adequate fluid intake, or at risk of dehydration. ● Failure to respond to antiviral treatment already given for this episode.
Cautions	● Elderly patients, meaning those aged 65 years and over. Renal function declines with age, often without a formal diagnosis of chronic kidney disease, and all three antivirals carry a higher risk of neurological adverse effects in this group, including confusion, hallucinations and somnolence. Check renal function before supply. ● Other nephrotoxic medicines, including ACE inhibitors, angiotensin receptor blockers, diuretics, NSAIDs, metformin, aminoglycosides and methotrexate. Supply may still be appropriate but counsel firmly on maintaining fluid intake. ● Tenofovir. Supply may proceed, but advise the patient to contact the prescriber of their tenofovir about additional renal monitoring. ● Probenecid or cimetidine. These reduce renal clearance of aciclovir and valaciclovir. The therapeutic index is wide so this is usually not significant, but it matters more where renal function is already reduced. ● Raloxifene with famciclovir. Raloxifene inhibits the enzyme that converts famciclovir to its active form, so antiviral effect may be reduced. Monitor the clinical response. ● Non-severe immunosuppression. Valaciclovir and famciclovir are both licensed for herpes zoster in immunocompromised adults; aciclovir is not licensed in the same terms. Where a patient has non-severe immunosuppression, use valaciclovir or famciclovir rather than aciclovir.
Actions if patient is excluded or declines treatment	● Discuss the reason for exclusion with the patient and ensure they understand it. ● Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. ● Document the reason for exclusion, the advice given and the decision reached. ● Inform or refer to the GP as appropriate, and where the exclusion

	is time critical make the urgency of that referral explicit to the patient.

Details of the medicine

Name, form and strength of medicine	• Aciclovir 800 mg tablets or dispersible tablets. • Valaciclovir 500 mg film-coated tablets. • Famciclovir 250 mg or 500 mg film-coated tablets. • Choose one agent. Do not combine.
Legal category	POM
Storage	Store in the original container. Follow the storage conditions in the SPC for the product supplied.
Route/method of administration	Oral.
Choice of agent	• Aciclovir 800 mg five times daily is the cheapest option but requires five doses a day at roughly four hourly intervals, omitting the night dose. Adherence is the practical problem. • Valaciclovir 1000 mg three times daily is preferred where five times daily dosing is impractical: patients whose medicines are given by a carer, those already taking eight or more medicines a day, and those with non-severe immunosuppression. • Famciclovir 500 mg three times daily is an alternative with the same adherence advantage as valaciclovir. It is not part of the NHS Pharmacy First service, so it is most relevant to private supply.
Dose and frequency of administration	• Aciclovir: 800 mg five times daily at approximately four hourly intervals, omitting the night dose, for 7 days. • Valaciclovir: 1000 mg three times daily for 7 days. • Famciclovir, immunocompetent: 500 mg three times daily for 7 days. • Famciclovir, non-severe immunosuppression: 500 mg three times daily for 10 days, which is the famciclovir SmPC course for herpes zoster in immunocompromised adults. • Start treatment as soon as possible. The benefit falls away the longer the delay after rash onset.
Renal function thresholds	**Do not attempt to operate the full renal dosing ladder in a community pharmacy. Where renal function falls below the thresholds below, refer to a prescriber.** • Aciclovir: do not supply under this PGD where eGFR is below 30 mL/min/1.73m². • Valaciclovir: do not supply under this PGD where eGFR is below 60 mL/min/1.73m². This is deliberately more conservative than the aciclovir threshold and matches the national Pharmacy First PGD. • Famciclovir: do not supply under this PGD where eGFR is below 60 mL/min/1.73m². The famciclovir SmPC gives the full dose of 500 mg three times daily only at creatinine clearance 60 ml/min or above and reduces the dose below that, so 60 is the point at which the SmPC dosing ladder begins; it is also the valaciclovir threshold in this PGD and in the national Pharmacy First PGD. • Where renal function is unknown and the patient is elderly (aged 65 years and over) or has risk factors for renal impairment, refer rather than assume.

Quantity to be administered and/or supplied	One complete course as specified above. No repeat supply under this PGD.
Adverse effects	Common to all three: headache, dizziness, nausea, vomiting, diarrhoea, abdominal pain, rash and pruritus. Aciclovir and valaciclovir can cause photosensitivity. Uncommon or rare across the class: confusion, hallucinations, agitation, tremor, ataxia, reduced consciousness, convulsions, encephalopathy and coma, most often in renal impairment or the elderly; acute renal failure; anaphylaxis; angioedema; blood dyscrasias. Valaciclovir and famciclovir have both been associated with DRESS, and famciclovir with serious skin reactions including Stevens-Johnson syndrome and toxic epidermal necrolysis. Consult the current SPC for the product supplied.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP with whom the individual is registered ● name of healthcare practitioner ● the date of rash onset and which treatment window applies ● the dermatome affected and the pain score recorded on the 0 to 10 scale ● that red flags were assessed and found absent, and that ophthalmic involvement was specifically excluded ● renal function as eGFR in mL/min/1.73m2 (the measure used by the thresholds in this PGD), and how it was established ● name and brand of medicine, dose, quantity and batch number ● advice given, including advice given if excluded or if the patient declines ● details of any adverse reaction and the action taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Adults aged 18 and over: keep records for 8 years.

Pain, infection control and safety netting

Pain

- For mild pain, advise paracetamol alone or with codeine, or an NSAID such as ibuprofen, subject to the usual contraindications.
- Refer urgently to a prescriber if pain is not controlled by over-the-counter analgesia.
- Explain post-herpetic neuralgia: pain persisting after the rash heals, more common with increasing age. It is managed with neuropathic pain treatments that require a prescription, so the patient should see their GP if pain persists.

Infection control

- The patient can transmit chickenpox, not shingles, to someone who has never had chickenpox or the varicella vaccine. Transmission is by direct contact with vesicle fluid.
- They remain infectious until all the vesicles have crusted over, usually 5 to 7 days after the rash appears.
- Advise them specifically to avoid pregnant women who have not had chickenpox, babies under one month old, and anyone who is immunosuppressed.

- Practical advice: cover weeping lesions that are not under clothing, keep the rash clean and dry, wear loose clothing, wash hands often, do not share towels or clothes, and avoid topical creams and adhesive dressings.
- Work, school and childcare need only be avoided while the rash is weeping and cannot be covered.

Safety netting

- Seek medical advice if the shingles has not resolved within 4 weeks.
- Seek advice if symptoms worsen significantly at any point, or do not improve after finishing the course.
- Seek advice if new vesicles are still appearing after 7 days of treatment.
- Seek advice for any new eye symptom, however minor.
- Seek advice if pain is not controlled.
- Call 999 or go to A&E for signs of sepsis, confusion, neck stiffness, weakness or loss of bladder or bowel control.
- Maintain a good fluid intake throughout the course, particularly if elderly.
- Once recovered, discuss the shingles vaccine with the GP practice. This is separate from treatment and is covered by a different PGD.

Patient information

- Give the patient the manufacturer's patient information leaflet.
- Explain the dosing schedule clearly and, for aciclovir, that five doses a day is demanding and the course will not work well if doses are missed.
- Give the infection control and safety netting advice above and record that it was given.
- Advise that antivirals reduce the severity and duration of the episode but do not cure it instantly, and that some pain may persist.
- Advise the patient to return any unused medicine to a pharmacy for disposal.

References

- NICE Clinical Knowledge Summary. Shingles. Current version.
- NHS England. Pharmacy First service, shingles clinical pathway and the national PGDs for aciclovir and valaciclovir. Current versions.
- Summary of Product Characteristics for aciclovir, valaciclovir and famciclovir, current versions, electronic Medicines Compendium.
- UKHSA. Immunisation against infectious disease, the Green Book, chapter 28a, for the definition of severe immunosuppression.
- British National Formulary, current edition.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	New PGD. Created because the shingles treatment entry in the catalogue was serving the Shingrix vaccine document, which contains no antiviral. A pharmacist opening it expecting treatment guidance found a vaccination PGD. Found on 21 August 2026 during a fingerprint of every document in the catalogue, prompted by the Pharmacy Plus Health query about the combined travel PGD. Renal thresholds follow the national Pharmacy First PGDs rather than the full SPC dosing ladders, on the basis that titrating renal doses is a prescriber task. The famciclovir threshold is a house position by analogy, as no national PGD exists for it.	21st August 2026

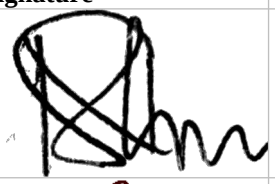
Version and change record
Version: v009
Issued: 30 September 2026
Supersedes: Version 008, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v008
Issued: 14 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 18: the 72 hour inclusion criterion now reads aged 50 years or over, matching NICE CKS as summarised in part 2; it previously read age over 50.
Decision 19: moderate or severe pain is defined as a score of 4 or more on a 0 to 10 scale and severe pain as 7 or more; elderly is defined as 65 years and over in the cautions and renal threshold rows; the training and records rows name the 0 to 10 scale.
Decision 20: sacral dermatomes, including the buttocks, are stated to count as truncal, so sacral involvement does not meet the non-truncal criterion of the 72 hour window.
Decision 22: breastfeeding is an exclusion in every case, referred to a prescriber, in line with the NICE CKS advice quoted in part 2; the previous distinction between sores on the breast and sores elsewhere is removed.
Decision 23: the famciclovir SmPC (Famvir 500 mg film-coated tablets, Phoenix Labs, text revised 18 March 2019) is fetched and summarised in part 2 (dose and indication, renal dosing table, elderly, interactions, hepatic impairment, contraindications, pregnancy and breastfeeding, adverse effects and zoster warnings). The eGFR 60 threshold is confirmed against it: the SmPC gives the full 500 mg three times daily dose only at creatinine clearance 60 ml/min or above and reduces the dose below that, so the renal threshold row no longer describes it as an analogy. The 10 day course in non-severe immunosuppression is confirmed as the SmPC regimen for herpes zoster in immunocompromised adults.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Cover page narration about the catalogue slug removed; the cover now states only that this is a treatment PGD and that Shingrix is covered by the separate vaccination PGD.
Records row now states that renal function is recorded as eGFR in mL/min/1.73m2, the measure the PGD thresholds use.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v002
Issued: 10 September 2026
Supersedes: Version 001, 21 August 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v003
Issued: 10 September 2026
Supersedes: Version 002, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Indication now covers immunocompetent adults AND adults with non-severe (mild or moderate) immunosuppression, treated with valaciclovir or famciclovir, which the previous version already dosed and cautioned for while its indication said immunocompetent only. Severe immunosuppression (Green Book chapter 28a) remains an exclusion.
Head or neck involvement is an exclusion. The inclusion criterion listed 'non-truncal involvement of the neck' as a reason to supply while the guidance summary (NICE CKS) says head and neck involvement is a trigger for admission or specialist advice.

Signed on behalf of Get Real Health

Version v009, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.